

QUALITATIVE AND QUANTITATIVE COMPOSITION

Each sachet contains Paracetamol 250 mg.

PRODUCT DESCRIPTION:

A dry, loose, orange effervescent powder with scent. When dissolved in water, the powder effervesces to produce an orange yellow, clear or slightly opaque solution with scent and slight sweet.

PHARMACODYNAMICS

Paracetamol is an effective analgesic and antipyretic agent. The drug has no effect on the cardiovascular and respiratory systems, and it does not cause gastric irritation or bleeding like salicylates.

PHARMACOKINETICS

Absorption

The absorption of paracetamol by the oral route is complete and rapid. Maximum plasma concentrations are reached 30 to 60 minutes after ingestion

Distribution

Paracetamol is distributed rapidly in all tissues. Concentrations are comparable in blood, saliva and plasma. Plasma protein binding is low.

Metabolism

Paracetamol is metabolized primarily in the liver. The two major metabolic pathways are glucuroconjugation and sulphoconjugation. The latter pathway is rapidly saturable at dosages above therapeutic doses. A minor route, catalyzed by cytochrome P450, is the formation of a reactive intermediate (N-acetyl benzoquinone imine), which under normal conditions of use, is rapidly detoxified by reduced glutathione and eliminated in the urine after conjugation with cysteine and mercaptopuric acid. On the other hand, during mass poisoning, the quantity of this toxic metabolite is increased.

Elimination

The elimination is essentially urinary. 90% of the ingested dose is eliminated by the kidney in 24 hours, mainly in glucuroconjugate (60-80%) and sulfoconjugate (20-30%).

Less than 5% is eliminated as unchanged.

The elimination half-life is about 2 hours.

Pathophysiological changes

- Renal insufficiency: In patients with severe renal insufficiency (creatinine clearance less than 10 ml/min), the elimination of paracetamol and its metabolites is delayed

- Elderly: The conjugation capacity is not modified.

INDICATIONS

For the relief of mild to moderate pain and fever.

HAPACOL Junior Effervescent Powder 250mg

RECOMMENDED DOSE

Child:

- 2 to < 6 years or 14 kg to < 17 kg: 250 mg (1 Sachet) per dose. Repeat every 6 hours when necessary. Maximum of 750 mg (3 Sachets) in 24 hours.

- 2 to < 6 years or 17 kg to < 21 kg: 250 mg (1 Sachet) per dose. Repeat every 6 hours when necessary. Maximum of 1000 mg (4 Sachets) in 24 hours.

- 6 to < 8 years or 21 kg to < 25 kg: 250 mg (1 Sachet) per dose. Repeat every 4 hours when necessary. Maximum of 1250 mg (5 Sachets) in 24 hours.

- 6 to < 8 years or 25 kg to < 27 kg: 250 mg (1 Sachet) per dose. Repeat every 4 hours when necessary. Maximum of 1500 mg (6 Sachets) in 24 hours.

- 8 to < 10 years or 27 kg to < 34 kg: 500 mg (2 Sachets) per dose. Repeat every 6 hours when necessary. Maximum of 1500 mg (6 Sachets) in 24 hours.

- 8 to < 10 years or 34 kg to < 41 kg: 500 mg (2 Sachets) per dose. Repeat every 6 hours when necessary. Maximum of 2000 mg (8 Sachets) in 24 hours.

- 10 to < 12 years or 41kg to < 46 kg: 500 mg (2 Sachets) per dose. Repeat every 4 hours when necessary. Maximum of 2500 mg (10 Sachets) in 24 hours.

- 10 to < 12 years or 46 kg to < 50 kg: 500 mg (2 Sachets) per dose. Repeat every 4 hours when necessary. Maximum of 3000 mg (12 Sachets) in 24 hours.

ROUTE OF ADMINISTRATION: Oral.

CONTRAINDICATIONS

Hypersensitivity to paracetamol or other constituents.

Hepatocellular insufficiency.

Phenylketonuria (due to the presence of aspartame).

WARNINGS AND PRECAUTIONS

Unsuitable for phenylketonurics.

To avoid a risk of overdose, check the absence of paracetamol in the composition of other medicines.

Recommended maximum doses:

- The maximum daily dose of paracetamol is 60mg/kg/day, divided into 4 or 6 doses.

- In children, the total dose of paracetamol should not exceed 60mg/kg/day.

- In children over 50 kg, THE TOTAL DOSE OF PARACETAMOL MUST NOT EXCEED 4 GRAMS PER DAY (see Overdose).

Precautions for use

In children treated with 60 mg/kg/day of paracetamol, the combination of another antipyretic is justified only in cases of ineffectiveness.

Each sachet contains sodium and may be harmful to people on a low sodium diet.

This preparation contains PARACETAMOL. Do not take any other paracetamol containing medicines at the same time.

Allergy alert:

Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash.

These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

INTERACTIONS WITH OTHER MEDICAMENTS

Associations subject to precautions for use

+ Oral anticoagulants

Risk of increased effect of oral anticoagulant and hemorrhagic risk if paracetamol is taken at maximum doses (4 g/d) for at least 4 days. Regular monitoring of INR. Possible adaptation of the dosage of the anticoagulant during treatment with paracetamol and after discontinuation.

Interaction with paraclinic examinations

Taking paracetamol may distort the blood glucose test by the glucose oxidase-peroxidase method in abnormally high concentrations.

Taking paracetamol can distort the dosage of blood uric acid by the phosphotungstic acid method.

STATEMENT ON USAGE DURING PREGNANCY AND LACTATION

Pregnancy

Epidemiological studies in human pregnancy have shown no ill effects due to paracetamol used in the recommended dosage, but patients should follow the advice of their doctor regarding its use.

Breastfeeding

Paracetamol is excreted in breast milk but not in a clinically significant amount. Available published data do not contraindicate breast feeding.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES:

Not applicable.

UNDESIRABLE EFFECTS

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/ Toxic Epidermal Necrolysis have been reported.

A few rare cases of hypersensitivity reactions such as anaphylactic shock, erythema, urticaria have also been reported. Their occurrence requires the discontinuation of this medicinal product and related medicinal products.

Very rare cases of thrombocytopenia, leukopenia and neutropenia have been reported.

OVERDOSE AND TREATMENT: Poisoning is to be feared in the elderly and especially in young children (therapeutic overdose or frequent accidental intoxication) in which it can be fatal.

Symptoms

Nausea, vomiting, anorexia, pallor, abdominal pain usually appear within the first 24 hours.

Overdosage with 10 g of paracetamol as a single dose in adults and 150 mg/kg of body weight as a single dose in children results in hepatic cytolysis resulting in complete and irreversible necrosis and also causes hepatocellular insufficiency, metabolic acidosis, encephalopathy which can lead to coma and death.

At the same time, there is an increase in hepatic transaminases, lactic dehydrogenase, bilirubin and decreased prothrombin levels that may occur 12 to 48 hours after ingestion.

Emergency

- Immediately transfer to hospitals

- Take a tube of blood to make the initial plasma dose of paracetamol

- Rapid evacuation of the product ingested by gastric lavage.

- Treatment of overdosage typically involves administration as early as possible of the N-acetylcysteine antidote IV or orally if possible before the tenth hour.

STORAGE CONDITIONS: Store in dry places, not exceeding 30°C, protect from light.

DOSAGE FORM AND PACKAGING AVAILABLE

Effervescent powder. Box of 24 sachets x 1.5 g

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Manufactured by:

DHG PHARMACEUTICAL JOINT-STOCK COMPANY - DHG Pharmaceutical plant branch in Hau Giang

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